

East Bay Pain Management

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PROCEDURE NOTE

Patient: David Park
DOB: 09/24/1986
Account: EBPM-118
Date of Procedure: 09/09/2024
Procedure Start: 09:42
Procedure End: 10:08
Performing Provider: Sandra Liu, MD — Interventional Pain Medicine
Referring Provider: Reza Tehrani, MD

PRE-PROCEDURE DIAGNOSIS

Right L5 radiculopathy secondary to right paracentral disc protrusion at L4-L5 with nerve root impingement (per MRI 07/01/2024). Symptoms persistent despite 6 weeks of physical therapy and NSAID therapy.

PROCEDURE PERFORMED

Right L4-L5 transforaminal epidural steroid injection under fluoroscopic guidance.

INDICATIONS

Patient was seen in clinic on 08/28/2024 for evaluation. He was a candidate for transforaminal epidural steroid injection given imaging-confirmed pathology, dermatomally correlated symptoms, and failure of conservative management. Risks, benefits, and alternatives were discussed at that visit and again immediately prior to the procedure today. Written informed consent obtained.

PROCEDURE DETAILS

The patient was brought to the procedure suite and placed prone on the fluoroscopy table with appropriate monitoring (continuous pulse oximetry, blood pressure cuff). The right lower lumbar region was prepped and draped in standard sterile fashion. Time-out performed with all team members present, confirming patient identity, planned procedure, and laterality.

Under fluoroscopic guidance, the right L4-L5 neural foramen was identified using AP and oblique views with appropriate end-plate alignment. Local anesthesia (1 mL of 1% lidocaine) was infiltrated subcutaneously and along the planned needle trajectory. A 22-gauge, 5-inch Quincke spinal needle was advanced under fluoroscopic guidance into the right L5 neural foramen using a "safe triangle" approach. Position was confirmed in AP and lateral views.

Negative aspiration confirmed no blood or CSF return. 1 mL of Omnipaque-240 contrast was injected, with appropriate epidural spread along the right L5 nerve root visualized in real-time fluoroscopy. There was no vascular uptake. A solution of 40 mg triamcinolone (1 mL) and 1 mL preservative-free 1% lidocaine was then injected. The patient tolerated the procedure well without complications.

Sterile dressing applied. Patient was monitored in recovery for 30 minutes post-procedure with stable vital signs. Pre-procedure pain score: 5/10. Immediate post-procedure pain score: 4/10 (anesthetic effect).

POST-PROCEDURE PLAN

1. Patient discharged in stable condition with a responsible adult driver.
2. Standard post-injection precautions reviewed: avoid strenuous activity for 24 hours, keep dressing clean and dry, return precautions for fever, severe pain, weakness, or signs of infection.
3. Steroid effect expected to onset over 3-7 days, with peak effect at approximately 2 weeks.
4. Patient instructed to keep a brief pain diary for the next 4 weeks documenting pain ratings and functional status.
5. Follow-up in clinic in 4 weeks to assess response. If response is satisfactory, no further injections planned. If partial response, may consider repeat injection (up to 3 in a 12-month period).
6. Will communicate outcome with Dr. Tehrani.

Electronically signed:

Sandra Liu, MD

Board-certified, Anesthesiology and Pain Medicine

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